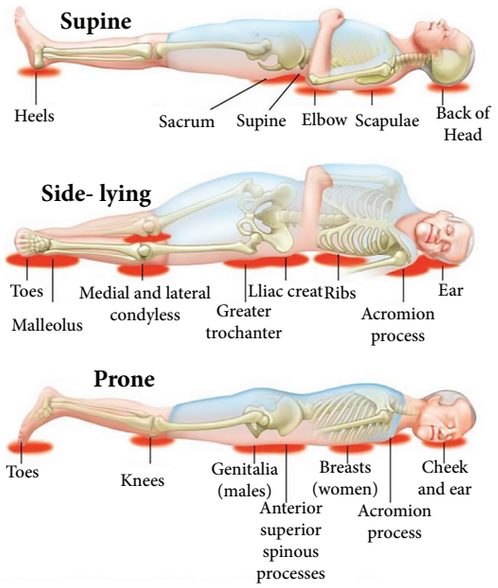
blister due to damage of underlying soft tissue from pressure and/or shear. The area may be preceded by tissue that is painful, firm, mushy, boggy, warmer or cooler as compared to adjacent tissue.

8.6.5 Areas Prone to Develop Pressure Sore



8.6.6 Risk Factors

Factors that may place a patient at risk include

- immobility,
- diabetes mellitus
- peripheral vascular disease
- malnutrition
- cerebro-vascular accident and hypotension.
- Other factors are age of 70 years and older,
- current smoking history,
- dry skin,
- low body mass index,
- urinary and fecal incontinence,
- physical restraints,
- malignancy, and history of pressure ulcers.

8.6.7 Prevention

8.6.7.1 Redistributing pressure:

The most important care for a person at risk for pressure ulcers and those with bedsores is the redistribution of pressure so that no pressure is applied to the pressure ulcer.

8.6.7.2 Support surfaces

Many support surfaces redistribute pressure by immersing and/or enveloping the body into the surface. Some support surfaces, including anti decubitus mattresses and cushions, contain multiple air chambers that are alternately pumped. Methods to standardize the products and evaluate the efficacy of these products have only been developed in recent years.

8.6.7.3 Nutrition

In addition, adequate intake of protein and calories is important. vitamin C has been shown to reduce the risk of pressure ulcers. People with higher intakes of vitamin C have a lower frequency of bed sores in those who are bedridden than those with lower intakes.

8.6.7.4 Treatment

The treatment includes the use of bed rest, pressure re distributing support surfaces, nutritional support, repositioning, wound care (e.g. debridement, wound dressings) and biophysical agents (e.g. electrical stimulation). Reliable scientific evidence to support the use of many of these interventions, though, is lacking.

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